



جوستاف روسي انترناشيونال - مصر (EPE) * طلب علاج أول مرة / متردد

إسم المريض ربيعاً : لمياء احمد عبدالمولى شراره
المحافظة : البحيرة
الحى / المدينة : كوم حماده
رقم البطاقة : 28406091800707 رقم التليفون : 01092280467
العنوان :
العمر (تاريخ الميلاد) : 09-06-1984 المهنة : ربه منزل
تاريخ تسجيل الإستمارة : 20-09-2026

الإقرار

أقر انا الموقع أدناه انني لا اتمتع باي نظام علاج مثل :-التأمين الصحي-القوات المسلحة-الشرطة-الهيئات-المؤسسات او اي نظام يتعارض مع نظام العلاج علي نفقة الدولة- وسوف اتحمل كافة نفقات العلاج في حالة ثبوت غير ذلك
الإحصائي الاجتماعي
التوقيع: (-----)
المقرر بما فيه
التوقيع: (-----)

التشخيص الإكلينيكي : سرطان بالثدي (مبادرة)
نتائج الأبحاث : (مرفق)
التشخيص النهائي : سرطان بالثدي (مبادرة)
خطة العلاج : علاج مبادرة سرطان الثدي علاج تدعيمي وتلطيفي [6579] قسم العلاج : الاورام
التعليق على التشخيص : -----

تقرير اللجنة الثلاثية المتخصصة

الاسم: د/محمد كيلاني
الاسم: د/ندى حسن
الاسم: د/نرمين رمضان
الوظيفة: -----
الوظيفة: -----
الوظيفة: -----
التوقيع: -----
التوقيع: -----
التوقيع: -----
أطلعت اللجنة علي لائحة العلاج الخاصة بالمجالس وتلتزم جهة العلاج بلائحة المجالس الطبية. كما تقرر اللجنة بان البيانات المدونة فى هذا النموذج صحيحة.
تعتمد صحة البيانات المرفقة
رئيس اللجنة ومدير
المستشفى.
تعهد المستشفى أن لا يتم تنفيذ القرار إلا بعد مراجعة الملف الطبي للمريض وإعادة مناظرة المريض بالمستشفى قبل الصرف.

أعضاء اللجنة

د. / ----- د. / ----- د. / ----- د. / -----

قرار اللجنة

التشخيص النهائي :
خطة العلاج :
المستشفى :
قسم :
جهة العلاج :
نفقات :
ملاحظات :

ختم
شعار الجمهورية

مدير عام
مديرية الشؤون الصحية

رئيس
المجلس الطبي المتخصص

--

Name: لمياء احمد عبدالمولى شراره National ID: 28406091800707	
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تقرير طبي

السيد
تحية طبية وبعد,,

إيماء للطلب المقدم من	بشأن تقرير طبي للمريض
بقسم:	رقم طبي:

نتشرف بان نوضح فيما يلي تقريرنا عن حالته
--

تاريخ وساعة الدخول : -- --:-- yyyy-mm-dd	تاريخ وساعة الخروج : PM 01:17 05-07-2026
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وبعد الكشف والفحوصات والمتابعة وجد انه
--

المریضة تعاني من ورم غير حميد بالثدى و تحتاج الي Supportive and palliative Treatment
--

التاريخ : 05-07-2026	الوقت : -- --:--
	الطبيب المعالج : د/شيماء لاشين



أشعة وفحوصات المرأة

Name | Mrs. Lamiaa Ahmad Abdelmawla
Date | 22-07-2026

■ **Breast Mammography and Complementary ultrasonography reveals:**

❖ **By Mammography:**

- Both breasts are ACR-b, scattered fibroglandular tissue.
- *Right breast LIQ shows three non-circumscribed speculated hyper dense lesions with internal micro calcifications.*
- *Right breast UOQ well circumscribed oval shaped hyper dense lesion.*
- No focal lesions seen at left breast.

❖ **By Ultrasonography:**

- Skin, nipple and areola on both sides are free.
- *Right breast LOQ at 5 o'clock non circumscribed hypoechoic speculated solid lesion measures about 3.2x2.5 cm surrounded with desmoplastic reaction with internal vascularity (RI=0.82). it seen 0.7 cm away from the skin and 1.8 cm away from the nipple. Another solid lesion sharing same criteria seen at 6 o'clock 8.4 cm away from the aforementioned lesion measures about 3x2.2 cm. It's seen invading the underlying muscles.*
- *Right breast UOQ at 11 o'clock zone A an intra mammary lymph node is noted with diffuse cortical thickening (3.3 mm).*
- *Right axilla shows multiple lymph nodes some show focal cortical thickening (6.3 mm).*
- Normal fibroglandular tissue distribution at left side.
- No pathological masses or cysts seen on left side.
- No dilated retroareolar ducts.
- Left axilla enlarged non specific lymph nodes with preserved fatty hila.

■ **Conclusion:**

- *Right breast LIQ multifocal malignant looking solid lesions as described with indeterminate intra mammary and right axillary lymph nodes for biopsy (BIRADS V).*
- *MRI with contrast is recommended.*
- *Normal left breast Sono-mammography examination (BIRADS I).*

Much obliged,

Dr. Shaza Ali

13 ميدان سفنكس - أعلى البنك الأهلي - شارع جامعة الدول العربية

f FB.com/HiaScan

instagram.com/HiaScan



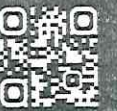
010 142 970 78



011 548 163 16



02 33 030 947



Mammogram

[[DOCTYPE:MAMMOGRAM]]

9/14/26, 4:46 PM

د. يوسف
روستام
أخصائي أمراض النساء



مبادرة رئيس الجمهورية
لدعم صحة المرأة المصرية

الهيئة العامة
للصحة
بمصر

09/14/2026

□

MDT FORM

Name: فهد أحمد حوالهول شازو	MRN: 13831
Age: 42 Y	ID: 28406091800707
Menopausal Sta	Phone: * 01226099617
NRS SCORE:	Marital Status: MARRIED
BMI:	Family History: -VE
Gender	Reason Of MDT: NEW CASE
CO Morbidity: FREE	ECOG Performance Status
Surgical History: * 2 CS	00 10 20 30 4

Clinical Data:

R

L

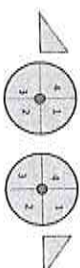
BX: IDC G2

ER: 8/8 PR: 8/8 HER2: 0 KI67: 10

History:

Sonomammography:

SM
RT BREAST LIQ MASS SOLID 3.5*2.5 CM SURROUNDED WITH
DESMOPLASTIC REACTION , 0.7 CM AWAY FROM THE SKIN AND 1.8 CM
AWAY FROM NIPPLE . ANOTHER SOLID LESION SHARING SAME CRITERIA
AT 6 O'CLOCK 8.4 CM FROM AFOREMENTIONED LESION 3*2.2 CM
INVADING MUSCLE . RT UOQ AT 11 O'CLOCK ZONE A IMLN
RT AXILARY LN



TNM

Biomarker/Molecular

Clinical Staging:

Mets Workup:
PET CT FREE

Pathological Staging:

Recommendations:
FOR POSSIBILITY OF NAHT

MDT Outcome & Plan

PORTA CATH + ECHO + CESM FOR SURGICAL DECISION
AC X 4 C /// TAXOL 12WKS
RECONSULT AFTER CESM FOR CLIPPING

SIGNATURE

MEDICAL ONCOLOGY	SURGERY	RADIOLOGY	PATHOLOGY
<i>[Signature]</i>	<i>[Signature]</i>		

about:blank

جمهورية مصر العربية
بطاقة تحقيق الشخصية

لمياء
احمد عبدالمولى شراره
١٣ ش الانصارى - عطفة داود العزب
الجيزة - الجيزة

١٩٨٤/٠٩
٢٨٤٠٦٠٩١٨٠٠٧٠٧

JU4081552





Gustave Roussy International - Egypt - Pathology Lab
جوستاف رويسى انترناتيونال - مصر - معمل الباثولوجى



Patient Name	اسم المريض	Case No.	345858
Sex/Age	Female 42 Y	Patient No.	25505
Registered Date	03/8/2026 03:21:18 PM	Referred By Dr.	Interventional Radiology
Validation Date	18/8/2026 02:37:49 PM	Branch	Gustave Roussy
Reporting Date	26/8/2026 11:18:18 AM	Pathology No.	2443/26

Pathology Report

Clinical Data: Right breast mass (BIRADS 5) diagnosed "Invasive duct carcinoma grade 2".

Nature of Specimen: Tru-cut biopsy, collected on 03/08/2026, at 1:45 pm. Cold ischemia time is 20 seconds.

Gross Examination:

- Three fragmented grey, white tissue cores, ranging in size between 0.2 cm & 1.0 cm long, totally submitted.

Technique:

- Sections prepared from paraffin block for immune-staining using Bench Mark ULTRA system for,
- ER clone (1E2), PR clone (SP1), Her2 clone (4B5), & Ki67 clone (30-9).

Microscopic examination:

- Histologic type: Invasive duct carcinoma.
- Histologic grade: 2 (differentiation 2, pleomorphism 2, mitosis 2).
- DCIS: Present, low grade, solid.
- Microcalcification: Absent.
- Angioinvasion: Absent.
- Perineural invasion: Absent.

Results:

* IPT (All controls showed appropriate reactivity):

- ER: Strong nuclear reaction in 90% of tumor cells (score 8/8).
- PR: Strong nuclear reaction in 90% of tumor cells (score 8/8).
- Her2: No membrane reaction in tumor cells (score 0).
- Ki67%: 10%.

Conclusion:

Right breast mass, Tru-cut biopsy:

- Invasive duct carcinoma, Grade 2.

IPT:

- ER: Positive (score 8/8).
- PR: Positive (score 8/8).



خريطة ورقية (4) - الدور الثانى - مبنى رقم (987) - كورنيش النيل - مصر (القاهرة) - مصر
Raoum No. (4) - First Floor-Building No. 987- Nile Corniche - Maser El Qadema - Cairo, Egypt

Patient Name	لمياء احمد	Case No.	345858
Sex/Age	Female 42 Y	Patient No.	25505
Registered Date	03/8/2026 03:21:18 PM	Referred By Dr.	Interventional Radiology
Validation Date	18/8/2026 02:37:49 PM	Branch	Gustave Roussy
Reporting Date	26/8/2026 11:18:18 AM	Pathology No.	2443/26

- Her2: Negative (score 0).

- Ki67%: 10%.

****Scoring system: "ER, PR"**

ASCO-CAP ER/PR Test Guideline Recommendations: proportion of cells staining (0, no staining, 1, >1%, 2, between 1% and 10%, 3, between 11% and 33%, 4, between 34% and 66% and 5, between 67%-100% of the cells staining). Intensity of positive tumor cells (0, none; 1, weak; 2, intermediate; and 3, strong). Total score from 0 to 8.

**** Scoring system: "Her2"**

Reference: ASCO-CAP Her2 Test Guideline Recommendation. IHC 0 (Null): as defined by no staining observed. IHC 0+ (Ultra-low) as defined by membrane staining that is incomplete and is faint/barely perceptible and within < 10% of the invasive tumor cells. IHC 1+: as defined by incomplete membrane staining that is faint/barely perceptible and within > 10% of the invasive tumor cells.

IHC 2+: based on circumferential membrane staining that is incomplete and/or weak/moderate and within > 10% of the invasive tumor cells or complete and circumferential membrane staining that is intense and within < 10% of the invasive tumor cells. IHC 3+: based on circumferential membrane staining that is complete, intense > 10% of tumor cells.

Signature

Prof. Dr. Maha Guinei.



مركز الباثولوجي - جوستاف روسي انترنشنال - مصر، معمل الباثولوجي - القاهرة - مصر
Room No. (4) - First Floor-Building No. 987- Nile Corridor - Mistr El Qaderna - Cairo, Egypt



Name: **Lamiaa Ahmed Abd Elmawla**

DOS: **03/09/2026**

Ref. By: **Prof.Dr.**

Age: **y/o**

F-18 Fluorodeoxyglucose (FDG) PET/CT of the whole body:

❖ Clinical History:

- A 48 year old female with recent history of right breast LIQ multifocal lesions pathologically proven as IDC grade 2.
- PET/CT is requested for status evaluation.

Technique:

- Total body PET scan one hour after IV injection of 300 MBq of 18 F-FDG.
- MSCT performed using GE Discovery simultaneously and used for attenuation correction, anatomical localization and diagnosis.
- SUVmax; a Semi-quantitative analysis would be done for selected ROIs and the normal threshold is <3.

❖ Findings

- ❖ **FDG-avid right breast patchy glandular nodularities most evident at lower quadrant, eliciting SUV max up to 2.5.**
- ❖ **Unremarkable appearance of the left breast.**
- ❖ **Low grade FDG-avid right internal mammary lymph node, measuring ~13 mm with SUV max ~ 1.7.**
- ❖ **No metabolically active or pathologically enlarged cervical, axillary, supra-clavicular, hilar or other mediastinal lymph nodes could be detected.**
- ❖ **Tracer starved solitary small lung nodule at apical segment of left lower lung lobe. Otherwise clear both lungs.**
- ❖ **No pleural or pericardial collections.**
- ❖ **Normal liver, spleen, pancreas, kidneys, suprarenals, aorta, IVC and pelvic organs.**
- ❖ **There is no adenopathy or nodal hypermetabolism at the abdomen or pelvis.**



Name: Lamiaa Ahmed Abd Elmawla

DOS: 03/09/2026

Ref. By: Prof.Dr.

Age: Y/o

F-18 Fluorodeoxyglucose (FDG) PET/CT of the whole body:

- ❖ No ascites.
- ❖ The brain appears normal on CT, with no abnormal focal metabolic activity detected.
- ❖ FDG biodistribution is rather physiologic, with no significant blastic or lytic skeletal lesions could be seen on CT to account for metastatic disease to bone.

❖ **Impression :-**

FDG-PET/CT whole-body scan shows:-

- FDG-avid right breast patchy glandular nodularities most evident at lower quadrant. Likely neoplastic.
- Low grade FDG-avid right internal mammary lymph node, likely metastatic.
- Free rest of surveyed body from any other significant glucose avid foci.

Thanks for your referral,
Truly yours,

Dr. Mohammed Akl, Msc

Dr. Mostafa Elshafie, MD,

Dr. Abo-Bakr Hawary, MD,
Head of Nuclear Med. Department.



Name | Mrs. Lamiaa Ahmad Abdelmawla
Date | 22-07-2026

■ Breast Mammography and Complementary ultrasonography reveals:

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Dr. Shaza Ali

